A self-guided approach to recovery from **Generalised Anxiety**

Self-therapy Guidebook:

GENERALISED ANXIETY

FIRST EDITION

Greylight Psychology

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PLEASE NOTE:

The material within this resource is not intended as a replacement for one-on-one sessions with a qualified mental health professional.

If you are struggling with genuine mental health challenges, it is advised that you are properly assessed and provided with a relevant, evidence-based treatment plan by your attending practitioner.

If you are at risk of self-harm or suicide, please contact the emergency supports local to you, or present to your local emergency department.

Firstly, welcome!

If you're reading this, you've likely decided to make a significant change in your life, and more to the point, you've chosen to experiment with applying the techniques yourself. This is by no means an easy task, and you will likely encounter additional and unique challenges compared to those working alongside a professional, but you'll also encounter unique and exciting opportunities on this guided journey towards recovery and a greater degree of purpose and happiness.

It must be said that self-therapy is not intended to be a complete replacement of traditional face-to-face therapy with a qualified and experienced professional. Instead, the goal of this ebook is to provide you with an opportunity to experiment with learning and applying the skills taught in evidence-based settings to your own unique situation, and to provide some relief to our overburdened mental health system. More importantly, it seeks to provide relief to you, who might be struggling to find any psychologists local to you with reasonable enough availability.

A few notes on recovery

In psychology, recovery is a dynamic word that encapsulates the ebb and flow of progress through therapy. It is a 'moving' word, in that there are periods of growth and periods of relapse, but regardless, the journey towards a significant remission continues. Is there a point at which someone is considered to have 'recovered'? Possibly, but psychologists don't generally view recovery as a singular point or threshold that, once crossed, is never 'uncrossed'.

As with most issues in life, problems we thought were dealt with can resurface, and the disappointment or shame after reencountering such an issue can drag us down far quicker and deeper than necessary. This is not to say that we must live in fear of past challenges reemerging when we're tired or vulnerable, but that we should remain prepared and always within arm's reach of our tools and recovery strategies.

Given this, recovery is about 'two steps forward, and one step back', and it's vital that we make room for the human element on this journey. We're going to make mistakes, and that's okay. We're going to rebel against what we know we should do, and that's okay too. Ultimately, recovery is about getting up after each fall, dusting ourselves off and soldiering forwards towards what we know we deserve. Human beings, and our brains more specifically, are wholly resistant to change, but the task is not impossible – far from it. With enough will power, discipline and grit, we can start to see a healthier, more idealised quality of life take shape, and this is one of the most beautiful experiences life on this planet has to offer.

A typical course of therapy

A standard course of face-to-face therapy can last anywhere between 10 to 20 sessions, depending on the pace of treatment, client characteristics and the severity of symptoms. The structure of this ebook is divided into Modules, with each Module accounting for approximately 2 to 3 sessions in conventional therapy. While you don't have to space them out in this way, you are welcome to do so. You are also free to move as quickly, or as slowly, as you see fit, and if you have some experience with therapy for generalised anxiety, you may wish to skip straight to the content applicable to your needs and goals.

What is self-therapy?

As the name suggests, self-therapy is about you guiding yourself through the recovery journey. It takes the core principles, psychoeducation and strategies from traditional in-room therapy, and places them at your disposal, while guiding you steadily so as to avoid moving too quickly.

It differs from self-help in that the information in this ebook is rigourously validated and drawn from a large variety of scientific studies, and seeks to emulate the pace and 'feel' of a guided therapeutic course. It is practical, but also clinical, and will focus largely on what has demonstrated to work at reducing symptoms as opposed to what makes you feel 'warm and fuzzy' in the short-term. This is not to say that it's 'superior' to self-help – it's just a different approach, and each individual will likely develop a preference for one over the other.

Contact us!

If you have any questions regarding the material, you can always contact us at our instagram page via direct message: ***@greylight_psychology*** or via our email address: ***greylight_psychology@gmail.com***

We wish you the very best, and truly hope that you find great value in the pages to follow.

Contents

MODULE 1.1 - PSYCHOEDUCATION

- Overview of symptoms - **1**
- What causes generalised anxiety? - **2**
- The neurobiology of generalised anxiety - **3**
- Prevalence - **4**
- What is CBT? - **5**
- What is ACT? - **7**

MODULE 1.2 - BUILDING INSIGHT

- Progressive muscle relaxation (PMR) - **8**
- Mindfulness meditation - **9**
- Thought diary - **10**
- Values - **10**

MODULE 2.1 - BASIC ACT STRATEGIES

- Basic concepts and strategies - **1**5
- Defusion - **15 - 16**

MODULE 2.2 - BEHAVIOURAL ISSUES

- Experiential avoidance - **1**7
- Frustration tolerance - **18**
- Anxiety sensitivity - **19**

MODULE 3.1 - UNDERSTANDING WORRY

- Attentional biases - **24**
- What triggers worry? - **24**
- Low tolerance for uncertainty - **24**
- Positive beliefs about worry - **25**
- Facts vs. worry - **26**
- Worry period - **27**

Module 1.1 - Psychoeducation

GENERALISED ANXIETY

Overview of symptoms

As its name suggests, the array of concerns and triggers associated with generalised anxiety is often broad, focusing on a wide range of feared scenarios or situations. Typically, anxious preoccupations revolve around the wellbeing of family and friends, issues with work or study, and worldwide events, although the resulting anxiety is often chronic, disproportionate and excessive. For the presence of generalised anxiety to be clinically significant, that is, diagnosable by a professional, the symptoms of anxiety must be present almost every day for 6 months (see specific DSM-5 criteria below).

A significant issue for many individuals experiencing generalised anxiety is the rampant and seemingly uncontrollable worry that often coincides. Worry is, in fact, the defining attribute of this disorder, and aside from the debilitating anxiety and instances of all-consuming worry, there are many accompanying physical symptoms, including **nausea**, **restlessness**, **muscle tension**, **headaches**, **digestive issues**, and **excessive sweating**. It's also not uncommon for individuals experiencing generalised anxiety to experience sleep difficulties, with symptoms interrupting the ability to fall or stay asleep. Unfortunately, chronic poor sleep has been shown to exacerbate symptoms of anxiety, which, in turn, increases sleep difficulties – this is why addressing sleep is foundational to most treatments for almost all psychological disorders, and treatments for generalised anxiety are no exception.

What causes generalised anxiety?

As with most psychological disorders, mental health professionals view the development of anxiety from a 'stress-diathesis' perspective, which is to say that it's likely a combination of inherited/biological factors, and environmental factors. In initial sessions with a psychologist, you'll often be asked if there is a family history of mental illness or ill-health, which is simply their way of investigating the possibility of a genetic link. In many cases, individuals with family history do not develop issues, and this could be due the lack of a trigger significant enough to bring on symptoms of generalised anxiety. Research has shown a moderate genetic link for generalised anxiety, with genetic factors explaining approximately 30% of symptoms appearing. Interestingly, the presence of first-degree relatives (parents, offspring and siblings) with generalised anxiety predicts a twofold increase in the prevalence of internalising disorders, including anxiety, and a five-to-sixfold increase in the prevalence of generalised anxiety for other family members (Aktar et al., 2017).

Environmentally, our early experiences play a key role in the development and maintenance of generalised anxiety, with caregivers, specifically parents, having perhaps the greatest degree of influence. **Environmental transmission** refers to qualities in early developmental phases that shape a developing child's perception of threat, their self and the external world. A developing child who routinely observes their parents avoiding anxiety-invoking situations, fostering intolerance of uncertainty, and with an inability to cope with stress may go on to develop similar thinking and behavioural patterns in line with the principle of **modelling**, which accounts for why children model the behaviours observed around them.

Both verbal and non-verbal caregiver behaviours can influence a developing child's sense of safety as early as infancy, and infants will look to others for guidance on how to react to new stimuli. This tendency, referred to as **social referencing**, explains how the potential for disordered anxiety can begin very early in our development. This tendency continues well into toddlerhood, with studies indicating toddlers continuing this tendency to match their parent's anxious/fearful response towards neutral objects (Gerull and Rapee, 2002; Aktar et al., 2017). Without personal insight or the noted observations of concerned friends or family, we can begin to unconsciously ingrain behaviours and thinking patterns that promote and maintain disordered anxiety well into adolescence, and eventually adulthood.

The neurobiology of generalised anxiety

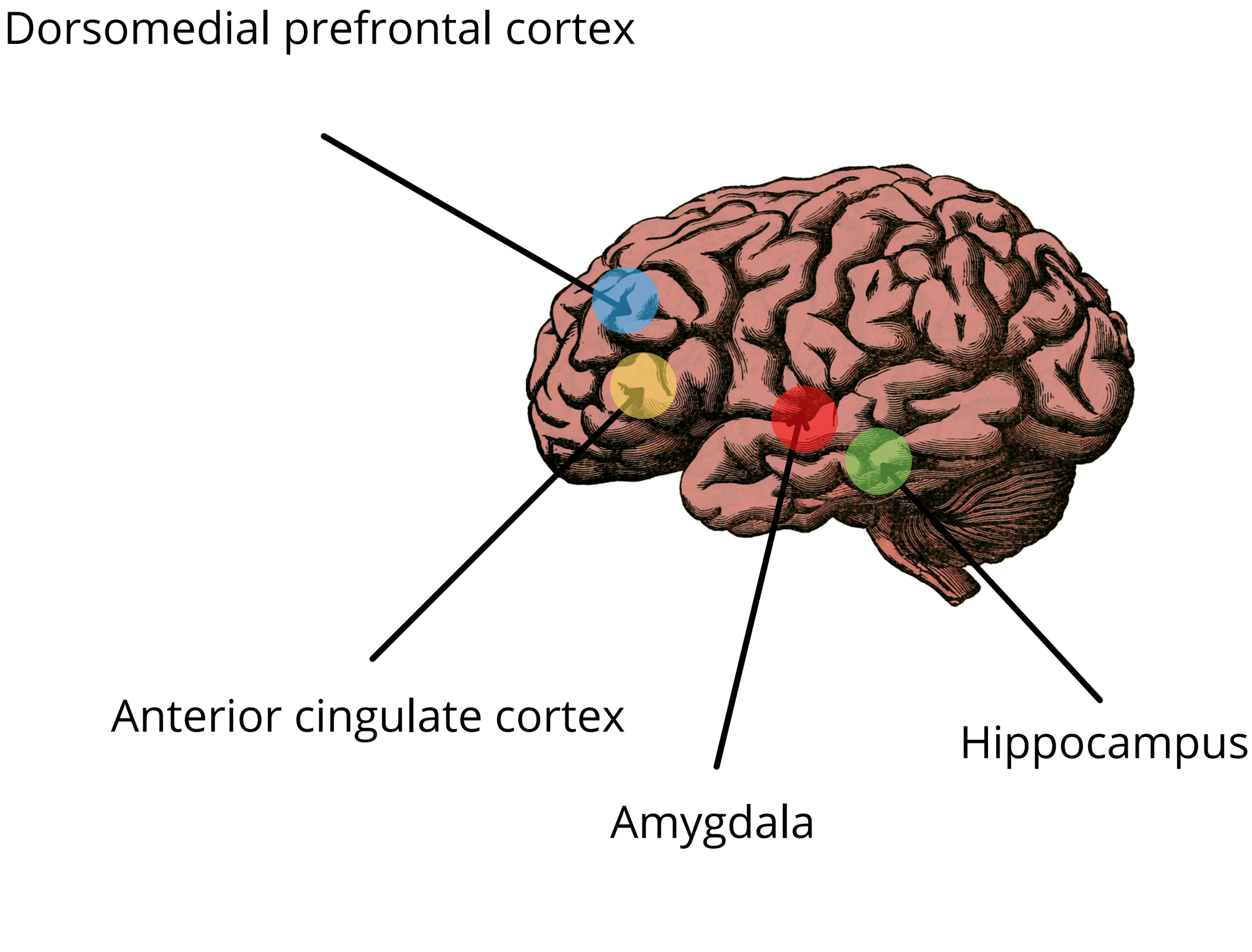
Neuroimaging studies of the brains of those experiencing generalised anxiety have revealed an overall increase in the size of a brain structure known as the **amygdala**. The amygdala is a complex structure of cells situated within the limbic system of the brain, and is thought to be responsible for our assessment and response to potential threats within our environment. Additionally, it regulates our fear-based and aggressive emotions, and is thought to connect emotions with memories. In animal studies where the amygdala is stimulated, aggressive behaviour is often the result. In generalised anxiety, there is evidence to suggest a 'hyperactivity' of this brain region, which is observed in other disorders, including PTSD, social anxiety, and specific phobia.

Another characteristic of the brains experiencing generalised anxiety is a larger **dorsomedial prefrontal cortex**, which is thought to be responsible for goal-directed behaviour, cost-benefit analysis and an awareness of our emotions and the emotions of others. In those with generalised anxiety, abnormal activity of this brain region is associated with less 'cognitive control', that is, less ability to reduce excessive anxiety using thought-based strategies.

If the situation was hopeless, psychologists would be out of work!

Now, all of this information regarding how anxiety manifests may make the situation seem hopeless, but I assure you it is not. What is learned can be unlearned, and you won't necessarily have to wait long before you start to notice this for yourself!

Figure1. Corticolimbic System



DSM-5 CRITERIA FOR GENERALISED ANXIETY DISORDER

1. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities.
2. The individual finds it difficult to control the worry.
3. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months):

   a. Restlessness, feeling keyed up or on edge
   b. Being easily fatigued
   c. Difficult concentrating or mind going blank
   d. Irritability
   e. Muscle Tension
   f. Sleep disturbances (difficulty falling or staying asleep, or restless, unsatisfying sleep)

4. The anxiety, worry, or physical symptoms cause significant distress or impairment in social, occupational, or other areas of functioning.
5. The disturbance is not attributable to the physiological effects of a substance or another medication.
6. The disturbance is not explained by another mental disorder.

DISCLAIMER: Diagnosis is beyond the scope of this ebook, and if you want to be properly assessed by a qualified professional, it is recommended that you book in an initial appointment with your local or family doctor. The DSM-5 criteria are included for reference, and should not be used as a means of self-diagnosis.

Prevalence

22% of all primary-care patients reporting anxiety are specifically diagnosed with generalised anxiety (Wittchen, 2002). Overall, females are more likely than males to experience generalised anxiety, and prevalence is highest (10%) amongst middle-aged women over the age of 35. Generalised anxiety is typically chronic, and follows a wax/wane course with few remissions. Fortunately, considerable evidence points to the very real potential of recovery when the correct forms of treatment, both pharmacological and psychological, are used.

COGNITIVE BEHAVIOURAL THERAPY (CBT)

What is CBT?

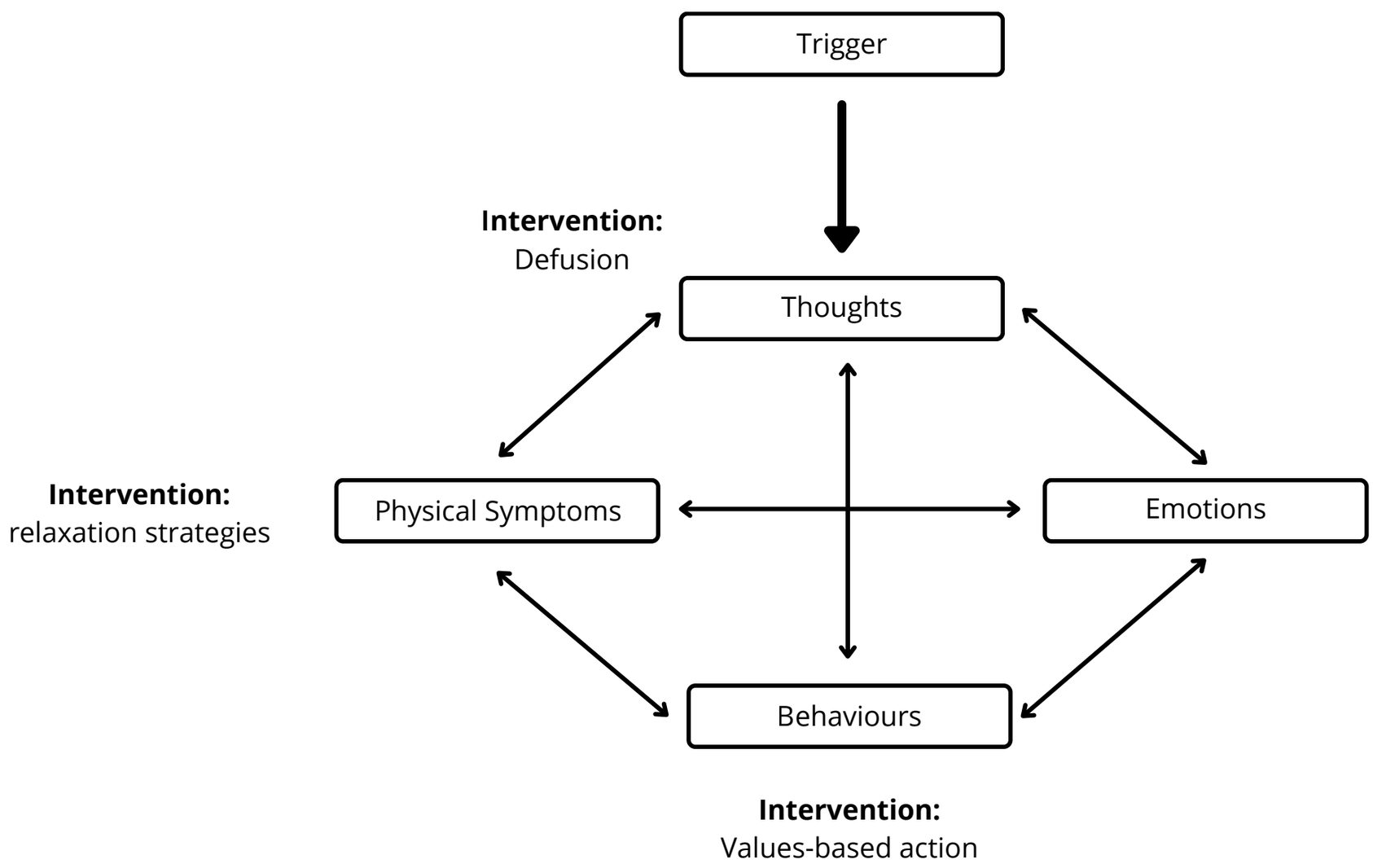
As a theoretical model, **Cognitive Behavioural Therapy (CBT)** has consistently demonstrated its capacity to conceptualise and reduce symptoms associated with generalised anxiety, and many other common psychological disorders. CBT is divided between three core components; **thoughts, feelings** and **behaviours**, and the model also takes into consideration the interrelatedness of these components.

Thoughts: thoughts arise in the form of perceptions, words, images, considerations and ideas, and are thought to contribute greatly to the emotions we will feel and the behaviours we engage in. They can emerge spontaneously, as is the case with automatic thoughts, or they can be the product of more enduring cognitive patterns, as is the case for intermediate or core beliefs.

Feelings: feelings are the emotions we feel in response to our interpretations of external and/or internal triggers, and can range from pleasurable to painful. While no feeling is inherently 'good' or 'bad', we as humans tend to have a preference for pleasurable emotions, and an avoidance of more painful feelings. In CBT, the way we think about these internal/external triggers produces certain emotional responses, which in turn prompt us towards a corresponding response or behaviour.

Behaviours: in CBT, behaviours, or our physical responses to internal/external triggers, are motivated or prompted by the emotions we experience, which are in turn guided by our thoughts and perceptions. A significant proportion of generalised anxiety recovery is behaviour-focused, with reductions in avoidance and worry behaviour being a major component of therapy.

Figure 2. The Cognitive Behavioural Model



As **Figure 2.** demonstrates, the CBT model does not flow in a singular direction, aside from the initial situational trigger impressing upon our conscious awareness. As we think, we feel, and as we feel, we behave, but our thoughts influence both our feelings and behaviours, just as our behaviour influences our thoughts and feelings. **Figure 1**. also details the targets for CBT strategies and techniques, namely our thoughts, behaviours, and our physical symptoms – the last of which can be particularly problematic in generalised anxiety.

The typical symptom profile of someone with generalised anxiety is observed below, and as you can see, generalised anxiety produces a wide range of negative effects on the mind and body.

The most obvious and common cognitive (thought) symptom is worry, and the most common corresponding feeling is anxiety. Behaviourally, avoidance is common as excessive anxiety leaves triggering situations feeling disproportionately threatening and dangerous. Naturally, we seek to avoid danger, and because the sense of danger is excessive and widespread for those of us experiencing generalised anxiety, the world and our lives can begin to feel constricting and severely limited.

Which of the symptoms below ring true for you?

Figure 3. The cognitive model and symptom profile of generalised anxiety

Cognitive	Feelings	Emotional
• Excessive worry • Negative and anxious thoughts • Focus on worst case scenario(s)	• Anxiety • Worry • Stress • Fatigue • Irritability • Guilt and shame • Increased chance of developing depression and other anxiety disorders	• Avoidance • Compulsive rituals • Procrastination • Inability to relax • Rumination

ACCEPTANCE AND COMMITMENT THERAPY (ACT)

What is ACT?

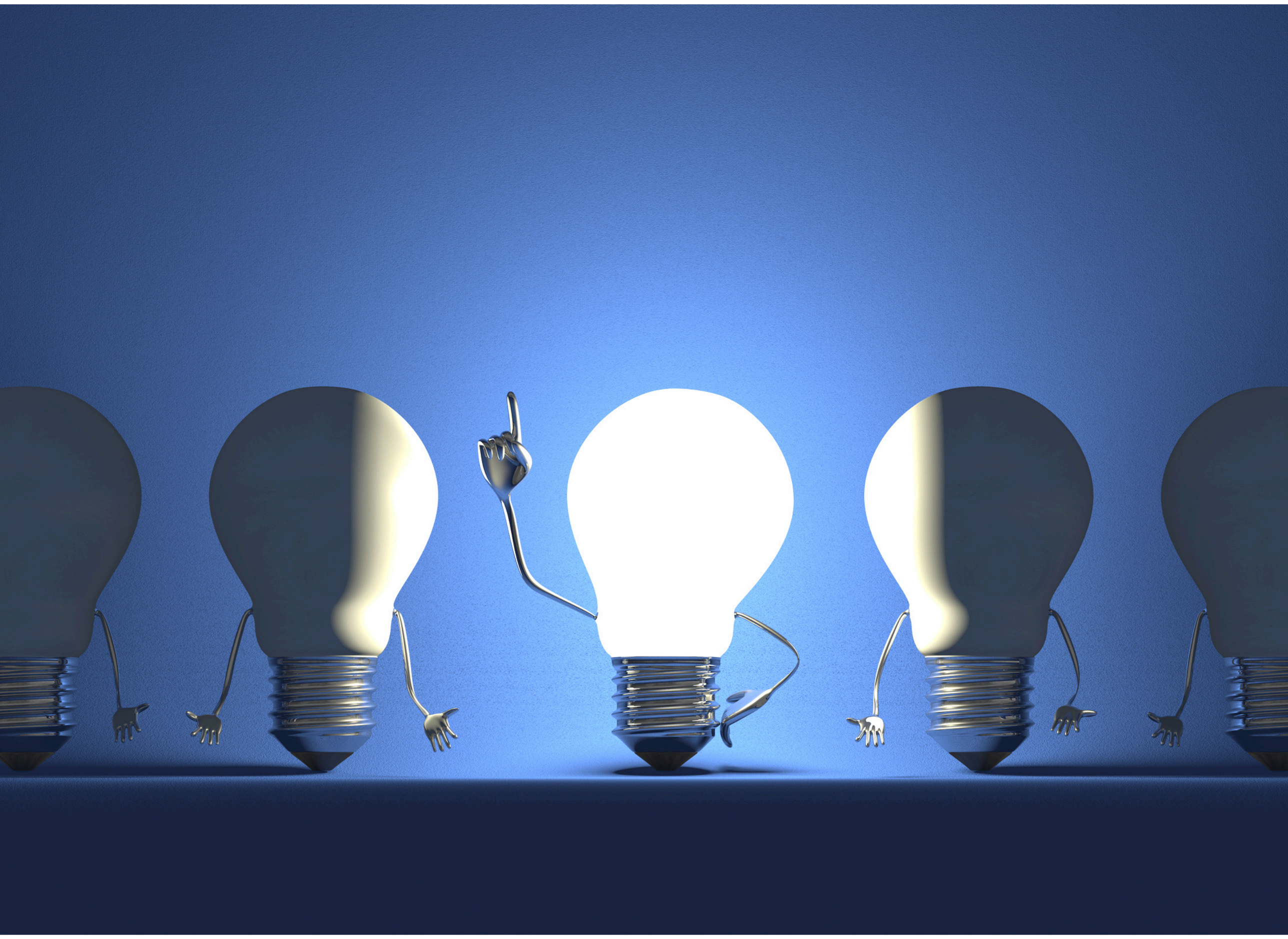
While we will be using the above theoretical model as a basis, we'll also be adding to it another model referred to as **Acceptance and Commitment Therapy (ACT)**.

ACT builds off the foundational theory of CBT, but has a different underlying philosophy and overall approach to 'treating' or dealing with anxiety, anxious thoughts, and the resulting behavioural issues.

ACT is divided between **acceptance** and **commitment**, with acceptance referring to the whole and total acceptance of all material present within our consciousness at a given time, be it thoughts, feelings, impulses or sensations. On the other hand, commitment refers to our capacity to make goal-directed and value-based decisions regarding our behaviours, regardless of what other material may be present within our awareness when we decide to take action. Whereas CBT prompts us to directly evaluate each thought, scrutinize it for evidence for and against, and to change it to a more rational or balanced alternative, ACT simply tells us to notice each thought, how it makes us feel and gently return our focus to the present before acting in accordance with our values.

While it might seem strange to do 'nothing' to try and change our way of thinking, ACT does not actually seek to change anything at all aside from our behavioural responses. It simply invites us to leave the mess of irrational thoughts as it is, and focus elsewhere. It views thoughts as meaningless chatter of the mind, based on outdated perspectives and beliefs, and directs us to 'dis-identify' from our thoughts each time they arise. In doing so repeatedly, we lessen our identification with them, thereby reducing their power over us. In ACT, the real mechanism for change lies in healthy, value-based actions, which bring us closer to the goals and objectives we set for ourselves and our lives.

While you will be taught a range of ACT techniques in considerable details, this Module will start by helping you to build an awareness of your automatic thinking, and thoughts arising from intermediate and/or core beliefs. In this way, you'll no longer need to concern yourself with the 'where and why' of your thought processes, which in itself can reduce overall fear and anxiety.



Module 1.2 - Building insight

MINDFULNESS

Mindful...what!?

Mindfulness refers to our capacity to remain aware of our internal experiences at any given time. It promotes an openness and curiosity to our thoughts and feelings guided by compassion and an increased willingness to embrace, rather than avoid, the material of our consciousness. Because mindfulness is indeed a skill, we must practice it regularly, and there are a number of ways in which we can build our capacity for it – some of which are detailed below:

Progressive Muscle Relaxation (PMR)

PMR involves the tensing and releasing of specific muscle groups throughout the body in a particular set sequence, and provides an external focal point away from excessive anxiety or worry. Furthermore, it can help alleviate symptoms of muscle tension, which are common to many anxiety disorders, including generalised anxiety.

- Firstly, minimize distractions in your environment – turn the TV off and use soft lighting.

- As you tense each muscle group, do so for approximately 5 seconds, ensuring you're not tensing to the extent that you're in pain.

- Relax the tensed muscle, and keep it relaxed for approximately 10 seconds

- Slowly rehearse the word 'relax' as you release each muscle group

The PMR sequence

1. Right hand and forearm. Make a fist with your right hand.
2. Right upper arm. Bring your right forearm up to your shoulder to “make a muscle”.
3. Left hand and forearm.
4. Left upper arm.
5. Forehead. Raise your eyebrows as high as they will go, as though you were surprised by something.
6. Eyes and cheeks. Squeeze your eyes tight shut.
7. Mouth and jaw. Open your mouth as wide as you can, as you might when you‘re yawning.
8. Neck. Be careful as you tense these muscles. Face forward and then pull your head back slowly, as though you are looking up to the ceiling.
9. Shoulders. Tense the muscles in your shoulders as you bring your shoulders up towards your ears.
10. Shoulder blades/Back. Push your shoulder blades back, trying to almost touch them together, so that your chest is pushed forward.
11. Chest and stomach. Breathe in deeply, filling up your lungs and chest with air.
12. Hips and buttocks. Squeeze your buttock muscles
13. Right upper leg. Tighten your right thigh.
14. Right lower leg. Do this slowly and carefully to avoid cramps. Pull your toes towards you to stretch the calf muscle.
15. Right foot. Curl your toes downwards.
16. Left upper leg. Repeat as for right upper leg.
17. Left lower leg. Repeat as for right lower leg.
18. Left foot. Repeat as for right foot

Mindfulness Meditation

Mindfulness meditation is a meditative practice that focuses one’s awareness on paced breathing and the present moment. It is simple in essence, but can be remarkably difficult initially.

- To meditate mindfully, sit somewhere comfortable, quiet and free from distractions, and with your back straight, free from tension or stiffness.

- If possible, breathe in slowly through your nose and into your stomach before exhaling through your nose or mouth.

- Notice the rising of your shoulders, belly and chest as you inhale, and the sensation of air entering and leaving your body.

- Naturally, your attention will drift to thoughts or sounds, and perhaps other internal experiences. Make allowances for this, and gently return your focus back to your breathing each time your mind wanders elsewhere.

- Repeat this practice for a time frame you’re comfortable with, and steadily increase the length of sessions as your confidence and focus expand.

Thought diary

No matter the context, we cannot solve a problem without a thorough understanding of the situation. This is all the more apparent for generalised anxiety recovery, as understanding the full scope of how it affects our thoughts, emotions and behaviour is key to reducing symptoms.

We will achieve this thorough understanding via the use of a thought diary, which is a record in which you will note down your initial emotions, thoughts and reactions to specific triggers, both internal and external. Doing this for even just a few days can bring enough insight to notice underlying patterns and themes between experiences, thereby increasing your understanding of yourself and your perceptual tendencies.

It is highly recommended that you print off the template on page **13**. You likely won't need the full page, depending on how long you intend to log your thoughts, but there is plenty of room provided if wish to continue the practice. It is advisable that you continue logging until you can notice the relationship between triggers, thoughts/interpretations, feelings and behavioural responses automatically and accurately. An example entry has been entered to demonstrate proper use of the thought diary.

Values

While the thought diary will help you develop insights into how you think, an understanding of your core values will provide you with a 'road-map' of how to respond to the cycle of triggers, thoughts and feelings. Values are our navigators through the unpredictability and challenges of life, and can be used to orient ourselves towards a consistent life path, even when experiencing great personal difficulty. They will act in this way throughout your recovery from generalised anxiety, and while we will not be covering how to use values in this module, it is vital that you decide upon what yours are.

There are a number of methods to aid you in deciding your value set, and you are welcome to use as many or as little as required. It is recommended that a value set consisting of between 2 and 4 values is most effective. Having too little may leave you with too few guiding principles, and too many can be confusing, particularly when deciding which value is most appropriate for a situation.

Values exercise 1

To determine your values, ask yourself the following questions:

- **When were you happiest?**
  - What were you doing?
  - Who were you with?
  - What was it about the situation that made you happy?

- **When were you most proud of yourself?**
  - Why were you proud of yourself?
  - Did anyone else share in your pride? If so, who?
  - What other factors contributed to your sense of pride?

- **When were you most fulfilled or satisfied?**
  - What need or desire was fulfilled?
  - What was/were the activity or activities?
  - What other factors contributed to your satisfaction?

After answering the 3 questions, attempt to match your responses with a corresponding value in the values list available on page **14**. If you find yourself with more than 4 values, see if you can combine values into a representative value. For example, you may combine 'helping society', 'justice' and 'charity' into a grouping value like 'compassion'.

Values exercise 2

Think about a real-life person or fictional character that you admire. What is it about them specifically that invokes your admiration? Note down their virtues and attributes, and if you are having difficulty doing so, confer to the values list on page **14** to decide which values apply to them. And remember; try to keep it between 2 and 4 values for conciseness' sake.

MODULE 1 SUMMARY

In this introductory model, you were introduced to generalised anxiety, and the many ways it can detract from our mental health and happiness. The manner in which generalised anxiety may arise from differences in the size and functioning of specific brain regions and structures was outlined, and the criteria required for diagnosis by a qualified professional were discussed.

Next, the CBT and ACT models were introduced, providing a rationale for their use and ways in which they can positively impact symptoms. The importance of insight by way of mindfulness was discussed, and a number of techniques were offered to help facilitate the development of the skills necessary to better understand the content and quality of your thoughts, emotions and behaviours.

In the next module, we will explore ACT in greater detail, and provide you with some foundational techniques to help disentangle your worrisome, anxiety-inducing thoughts from your identification with them.

Onwards!

MODULE 1 APPENDICES

Values list

Accountability
Accuracy
Achievement
Adventurousness
Altruism
Ambition
Assertiveness
Balance
Being the best
Belonging
Boldness
Calmness
Carefulness
Challenge
Cheerfulness
Clear-mindedness
Commitment
Community
Compassion
Competitiveness
Consistency
Contentment
Continuous Improvement
Contribution
Control
Cooperation
Correctness
Courtesy
Creativity
Curiosity
Decisiveness
Democraticness
Dependability
Determination
Devoutness
Diligence
Discipline
Discretion
Diversity
Dynamism
Economy
Effectiveness
Efficiency
Elegance
Empathy
Enjoyment
Enthusiasm
Equality Excellence
Excitement
Expertise
Exploration
Expressiveness
Fairness
Faith
Family-orientedness
Fidelity
Fitness
Fluency
Focus
Freedom
Fun
Generosity
Goodness
Grace
Growth
Happiness
Hard Work
Health
Helping Society
Holiness
Honesty
Honor
Humility
Independence
Ingenuity
Inner Harmony
Inquisitiveness
Insightfulness
Intelligence
Intellectual Status
Intuition
Joy
Justice
Leadership
Legacy
Love
Loyalty
Making a difference
Mastery
Merit
Obedience
Openness
Order
Originality
Patriotism Perfection
Piety
Positivity
Practicality
Preparedness
Professionalism
Prudence
Quality-orientation
Reliability
Resourcefulness
Restraint
Results-oriented
Rigor
Security
Self-actualization
Self-control
Selflessness
Self-reliance
Sensitivity
Serenity
Service
Shrewdness
Simplicity
Soundness
Speed
Spontaneity
Stability
Strategic
Strength
Structure
Success
Support
Teamwork
Temperance
Thankfulness
Thoroughness
Thoughtfulness
Timeliness
Tolerance
Traditionalism
Trustworthiness
Truth-seeking
Understanding
Uniqueness
Unity
Usefulness
Vision
Vitality

MODULE 1 APPENDICES

Thought diary

Trigger	Thoughts	Feelings	Behavioural response
Was the trigger internal or external?	**What automatic thoughts arose adter the trigger?**	**What feelings arose in response to the trigger and thoughts?**	**What course of action did you take?**
I made a mistake in a work meeting today	***Everyone will think I'm dumb and incompetent***	***Anxiety and shame***	***Called in sick the next day to avoid another meeting***

Module 2.1 - Basic ACT Strategies

ACT: BASIC CONCEPTS AND STRATEGIES

Introduction

In Module 1.2 you were introduced to Acceptance and Commitment Therapy (ACT), which differs from its forerunner, Cognitive Behavioural Therapy (CBT), in a number of key ways. CBT is an active, direct approach that seeks to identify, analyse and forcibly change problematic thoughts, whereas ACT adopts a more fluid and adaptable approach around the very same problematic thoughts to allow for more psychological flexibility and willingness to experience. The six core processes in ACT include; **Acceptance, Defusion, Contact with the Present Moment, Self-as-context, Values,** and **Committed Action** (Batten, 2011).

Most research studies and psychological manuals consider CBT to be the 'gold standard' therapy for almost any and all psychological disorders, or, at the very least, there is an appropriate adaption of the core model for most disorders, regardless of complexity. It is flexible, coherent and gets you, the client, involved in your own recovery. The reason why it is not used in the case of generalised anxiety is simple: you would likely burn out trying to critically analyse and find rational alternatives to each and every anxious thought that passes through your mind. It is called generalised, and not specific anxiety, for a very good reason.

CBT is excellent for individuals with limited philosophical problems relating to themselves, others or the world, and its highly targeted approach makes it suitable for challenging rigid core belief structures. For generalised anxiety, taking this approach would be ineffectual - akin to playing 'Whack'a'Mole' with your mind for all eternity, never quite getting the drop on those

annoying worrisome thoughts.

This is where ACT provides a much needed method for bypassing the chaos while still managing to effect meaningful and lasting change.

Basic concepts and strategies

ACT uses shifts in language and the way in which we conceptualise our thoughts to create this change. It's less about controlling thoughts, and more about controlling out reaction to them. Within the ACT model, thoughts are merely the biproduct of brain activity, and do not have much to offer by way of meaningful content. We are thinking machines, experiencing up to 6,200 thoughts a day, and ACT compels us to get into the habit of the 'watcher', someone who simply notices thoughts (and feelings) as they arise, greeting them with curiosity, kindness and openness. This might seem strange at first, but mastering the mindfulness techniques outlined in Module 1 will help you to understand and embody this process.

Returning to language shifts, replacing 'but' with 'and' can cement a small, but essential change in way we make sense of worrisome or anxiety-invoking thoughts. Since **behavioural restriction**, that is, our tendency to limit what we do to avoid anxiety, is apparent with generalised anxiety, changing our 'buts' to 'ands' can help us to move in the direction towards our chosen life path, sometimes dragging anxiety kicking and screaming along for the ride. Let us provide you with an example:

"I really want to go on this date, but I'm feeling anxious".

"I really want to go on this date, and I'm feelings anxious".

Notice how a single change in word provides an entirely different set of possibilities and attitudes? This is the goal and therapeutic response of ACT – we are encouraged to feel all that we do, and proceed forwards along our values-informed life path. Here's another example:

*"I need to go and do grocery shopping, **but** I feel really anxious".*

*"I need to go and do grocery shopping, **and** I feel really anxious".*

Again, switching the 'but' to an 'and' no longer places anxiety as an impediment or obstruction in the way of doing what we need to for ourselves. Get in the habit of doing the same, and you might find yourself less resistant to engaging in activities, and life in general, despite any accompanying anxiety.

Defusion

Another way in which ACT can help us to disentangle our identification from our thinking is through defusion. Defusion is the act (no pun intended!) of putting distance between our observing selves and the thoughts and feelings we encounter minute-by-minute. Perhaps the simplest way to summarise this practice is respond to the thoughts we encounter, rather than responding from them (Hayes, 2009).

Defusion technique 1: labeling

Labeling is the practice of placing an anxiety-invoking thought into a simple template that increases the separation between the observing self and the specific thought and associated feeling. It also helps to draw connections between our thoughts and feelings. The template looks something like this:

"I am noticing I am having the thought _____, and that this is resulting in me feeling the emotion of _____."

Let's take the prior example of a date to see it in action:

"I am noticing I am having the thought I'm unattractive, and that this is resulting in me feeling the emotion of depression".

As you can see, placing present thoughts and feelings into the above template helps to widen the gap between us and our internal

experiences, while also drawing a causal link between thoughts and feelings. Doing this routinely works to develop our insight and awareness of our internal states, and after considerable practice, you will like find you have more inner 'space' through which to feel more than just anxiety or worry.

Defusion technique 2: thanking the mind

ACT wants us to have fun and to be creative in the ways in which we defuse from our thoughts and emotions, and 'thanking the mind' is no exception to this. Simply put, this technique is about responding to worrisome or anxiety-invoking thoughts with a very basic response:

"Thank you, brain!"

While this can appear superficial (and perhaps even patronising!), its effect lies in stopping us from becoming caught up in the cycle of thought, anxiety and worry. It operates almost like a mental stop sign that we can flash whenever we need to interrupt the automatic worry cycle before it begins. Adding a sarcastic tone or inflection to this basic response can emphasise the unserious nature of your thoughts, no matter much anxiety accompanies them.

Defusion technique 3: Repeating the thought

Building on the (optional) sarcastic tone for technique 2, repeating the thought encourages us to repeat the content of our thoughts in a way that is comical and silly for the express purpose of delegitimising any perceived threat. For example:

We might have a flurry of worrisome thoughts relating to a job interview we attended recently, and while this would normally overwhelm someone ill-equipped to manage, using the repeating the thought empowers us to laugh at the 'doomsday thinking' that comes with generalised anxiety. So, instead of taking the thoughts seriously, we might say:

"I'm not going to get this job, which means my partner will think I'm a loser, I'll be kicked out of my home, and will be begging on the street come Christmas!" all the while doing so with as silly a voice as you can muster.

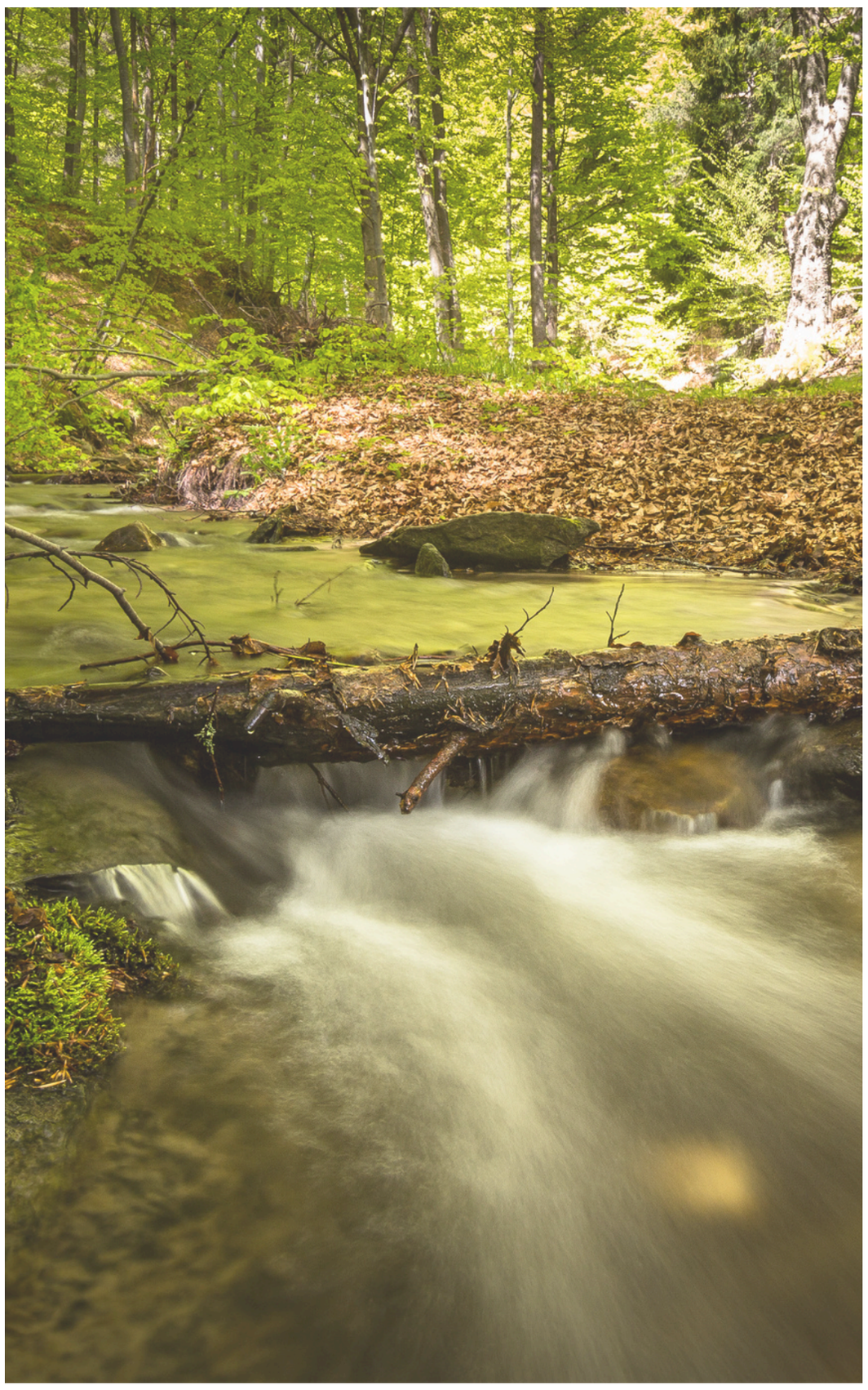
You're going to feel quite silly initially, but that's the point, because your thoughts will too! If you are feeling particularly confident, you might want to combine all 3 of the above techniques! Get creative and see what works for you and your unique situation.

Defusion technique 4: leaves on a stream

This technique calls upon the power of visualisation to gently, and gracefully, carry worrisome or anxiety-inducing thoughts away from our consciousness.

- Make sure you are sitting comfortably
- Envision yourself kneeling by a calm stream
- Imagine each thought as a leaf passing you by on this stream

Again, this might sound simple, but it does work.



Module 2.2 - Basic ACT Strategies

BEHAVIOURAL ISSUES IN GENERALISED ANXIETY

Experiential avoidance

Perhaps the greatest impediment to making genuine and lasting recovery with generalised anxiety, and arguably many other psychological disorders, is **experiential avoidance**. Fundamentally, this concept refers to the pervasive avoidance of both external and internal experiences, and as you can imagine, and possibly know first-hand, once anxiety and other difficult emotions dominate us in such a way that restricts what we can and cannot do, or where we go and do not go, our lives can become gloomy, caged in and suffocating. In circumstances such as these, we trade joy and spontaneity for a delicate sense of safety and control, only to find we develop new anxieties regarding our stagnation and perceived inability to confront our issues head-on. Some examples of experiential avoidance might include:

- Avoiding attending to the mounting housework due to the anxiety associated with starting
- Avoiding a shopping centre due to anxiety
- Avoiding feelings of grief by consuming alcohol
- Avoiding feelings of anxiety associated with university assignments by playing excessive video games

As you can see, avoidance can come in many forms, and the consequences can be devastating, often resulting in more unnecessary emotional suffering. We are usually presented with two options when encountering any obstacle or life issue – we can face the issue in its initial form as it presents, dealing with the associated difficulties and uncomfortable emotions, or we can choose to

procrastinate, or avoid, which buys us a very small window of temporary relief at the cost of additional anxiety, restlessness and negative consequences. ACT seeks to help break our tendencies to avoid, so we can flexibly tolerate discomfort and respond adaptively in a more timely fashion.

Remember the values exercises conducted in Module 1? This is where they come in to guide our adaptive responses, regardless of any anxiety we might feel. Using our values, we can construct more healthy responses to life issues and anxieties as they arise in the present moment.

One way to help bring these values into action is to brainstorm a number of activities related to each – this way, if you find yourself needing a commitment-based activity while heightened, you can simply refer to the table on page **22** to get started – it is ideal to have a combination of quick activities and more lengthy alternatives.

Frustration tolerance

Why do we avoid our problems knowing full-well we are only making the situation worse in the long-term? Humans can be remarkably short-sighted, particularly when seeking to avoid pain or discomfort, and frustration tolerance, or a lack thereof, can explain this phenomenon. It refers to the inability to cope when faced with stress and the frustrations associated with thwarted expectations, which increase our chances of responding in an unhealthy way (as evidenced in the example above). Research indicates a clear relationship between frustration tolerance and generalised anxiety, in that those who experience symptoms of generalised anxiety also report lower levels of frustration tolerance. This should not come as a surprise, as the more anxious we are, the more likely we are to react impulsively through avoidance and other unhealthy means. Lower frustration tolerance has also shown to increase the chances of major depressive episodes (Dennhardt and Murphy 2011; Anestis et al. 2012; Buckner et al. 2007; Ellis et al. 2010; Gorka et al. 2012).

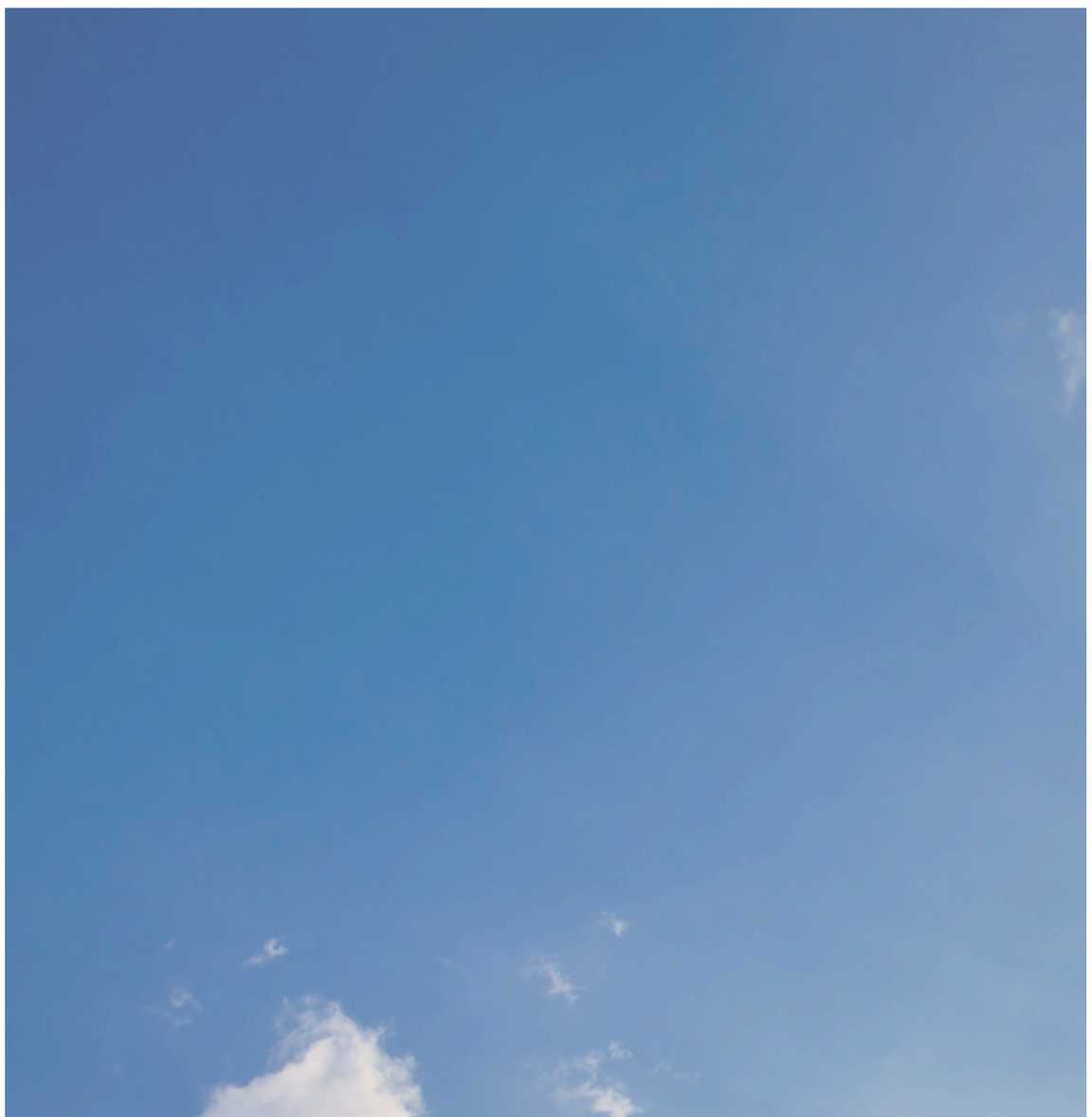
The great news is that frustration tolerance has been shown to be environmentally-mediated, which is to say that changes in one's environment, including changes made through therapy, can result in changes in one's capacity to tolerate distress or frustration (Bornovalova et al. 2012; McHugh et al. 2013). So, how do we build our capacity to deal with life's challenges and the challenges feelings that are bound to arise? We employ the defusion techniques in module 2.1 and then the values-based responses outlined in Module 1.2 – all of which is demonstrated in the helpful flowchart on page **21**.

This shows us that ACT is not concerned about getting rid of or changing negative and imbalanced thinking, but rather co-existing alongside it through dis-identification and observation. We can still get to where we need or desire to be by choosing behaviour in line with our personal values to achieve healthier life outcomes, which, in effect, will leave us feeling more accomplished and confident for future challenges, likely resulting in les experiences of generalised anxiety. A blank template for the above material will be available in this module's appendix section.

Anxiety sensitivity

Another concept closely associated to frustration tolerance is **anxiety sensitivity (AS)**. As unique individuals, we all possess differing tolerances for particular emotions and the amounts to which we can endure them. AS refers to the fear an individual experiences in relation to anxiety-related sensations, and is considered a risk factor for the development and maintenance of disordered anxiety, including generalised anxiety. There remains little doubt amongst researchers that AS is linked with pathological worry, or worry that is disproportionate to the cause of worrisome thoughts, and that its genesis lies in both hereditary and environmental factors. Not surprisingly, individuals diagnosed with generalised anxiety disorder (as per the DSM-5 criteria on page 4) tend to exhibit higher levels of AS, that is, they experience higher degrees of fear and resistance to anxiety-related symptoms (Allan et al., 2014).

Reducing anxiety sensitivity is most certainly possible, and utilizing the mindfulness and relaxation strategies outlined in Module 1 will help you to achieve this reduction. The greater our sense of mastery over regulating anxiety-related symptoms, the less apprehension and fear we'll experience when they do emerge.



MODULE 2 SUMMARY

In Module 2, the ACT model was expanded upon to provide you with a solid understanding of its core philosophy and how it seeks to alleviate symptoms of generalised anxiety. Defusion, which is the initial objective of ACT, was divided between four basic strategies that can be used as needed whenever difficult cognitive or emotional experiences arise.

Moreover, behavioural issues common to generalised anxiety were expanded upon, and remedies for each were provided. The Avoidance vs. Values-based Action table was explained in detail, and it streamlines the process by combining defusion and frustration tolerance into a single, practical technique.

In the next module we will return to CBT in order to evaluate the belief structures underpinning excessive worry, and any perceptions regarding what stopping the worry cycle might mean.

Onwards!

MODULE 2 APPENDICES

Defusion and values table

Trigger	Thought	Feeling	Avoidance Impulse	Consequence	Values-based response	Conseque nce
Due date for the next assignment is in 2 weeks	*"I'll never finish it on time"*	*Anxiety stress and worry*	*Play video games and worry about it later*	*Hand the assignment in late and poorly*	*Accept anxiety using defusion and use my value of commitment*	*Increased chance of on-time submissio n*

Values and activity table

Value	Activity

Module 3.1 - Understanding Worry

WHAT IS WORRY?

The basics

Worry can be defined as our tendency to fixate and ruminate upon issues of present or future concern. In and of itself, time-limited worry is a healthy response to distress or alarm, and is usually an extension of our concern or care, but in the case of pathological worry, this tendency extends beyond the bounds of what's observably proportionate or reasonable. Worry and anxiety are often experienced in tandem, with one prompting the emergence of the other, but worry is considered a distinct process in that anxiety refers specifically to feelings of tension and apprehension. Those who experience pathological worry tend to encounter repetitive negative thoughts from a variety of circumstantial triggers, and these thoughts generally take a *quasi-verbal form* – that is, they contain elements of language alongside visual or other imaginative elements.

At its core, worry is a means, and an often insufficient one, of trying to resolve or lessen the anxiety felt in relation to perceived threats in the environment by providing a sense of control or personal mastery. Evolutionarily speaking, the tendency to worry is likely to have emerged in order to aid us with overcoming obstacles via verbal mental problem solving that is short-lived and leads to healthy problem-solving behaviour. On the other hand, pathological worry is cumulative in that it builds upon itself, leaving less opportunity for more balanced or positive perceptions that may alleviate needless anxiety.

Worry is a component of many psychological issues, and it remains a key characteristic of generalised anxiety. It is has been found to interfere with the ways in which we interact

with our environment, leading to misattribution and attentional biases.

Attentional biases

Derived from the cognitive model of pathological worry (Hirsch and Mathews, 2012), attentional biases are the ways in which we skew or 'colour' our perceptions based on where our attention or focus lies. In the case of generalised anxiety, common attentional biases include a tendency to focus on perceived threats where there are none, or at the very least, to inflate the likelihood of a possible threat occurring. Neutral situations or circumstances that would elicit normal levels of threat perception in neurotypical individuals are more like to result in perceptions of threat for individuals experiencing consistently elevated anxiety levels.

For example, an individual who experiences generalised anxiety may be struck with worrisome thoughts regarding the wellbeing of their children throughout their school day. They may perceive the otherwise neutral context of the classroom or schoolyard as inherently threatening, or more threatening than it actually is. Bullies, domineering teachers, and the possibility of undiagnosed learning difficulties may cycle endlessly in the mind of an overly anxious parent, causing them significant and unnecessary distress.

What triggers worry?

There are no hard and fast rules for what triggers worry, as each individual holds a unique capacity to tolerate uncertainty and the prospect of negative outcomes occurring in the present or future. Anything can prove a cause for worry, particularly in the case of individuals experiencing disordered or excessive anxiety, but usually it is a combination of perceived situational stressors that initiates the worry cycle. Below is a common, but by no means exhaustive list of triggers for pathological worry in the context of generalised anxiety:

- **Disproportionate worry about the wellbeing of loved ones despite no evidence they are in danger.**
- **Disproportionate worry about losing employment despite positive comments on your recent performance review.**
- **Disproportionate worry about worldwide affairs that are beyond your control.**
- **Disproportionate worry that you will fail an upcoming assignment despite a history of good grades.**

From the above information, it is clear that some individuals harbour a tendency to identify threats, even in circumstances where no actual threat is apparent. So, what other factors contribute to a tendency to develop generalised anxiety and pathological worry? There are two broad characteristics that are considered relevant, and they are explained in detail below:

Low tolerance for uncertainty

While foundational fears can be difficult to pinpoint in generalised anxiety, it is almost guaranteed that intolerance of uncertainty will be playing at least a moderate role in the development and maintenance of symptoms. We have few guarantees in life, and remarkably little control over external events and how our daily experiences unfold, and to the anxiety-stricken individual, this generally, and reflexively, translates to a threat of imminent danger, whether that danger is physical, social or financial. Rather than viewing uncertainty as cause for proportionate caution, or even opportunity, generalised anxiety and pathological worry torments an individual, leaving them without a sense of safety, trust in themselves or others, and little hope for the future.

What, then, is the antidote? It might seem obvious, but developing a healthier, more realistic and proportionate relationship to uncertainty is key. While simple in essence, putting this into action can be agonizing, at least initially. To give up worry is to give up the only activity or response that provides any semblance of control over the unpredictability of life. In the warped perceptions of pathological worry, every single negative possibility must be accounted for, and worse, avoided at all costs (refer earlier to experiential

avoidance in **Module 1**) – it is no wonder that individuals with generalised anxiety consistently report exhaustion and fatigue, irritability and a dream-like state known as **depersonalisation**, which occurs when the mind simply 'checks out' due to excessive strain.

In order to achieve a healthier relationship with reality, we can recall the earlier strategies explored in Module 2.1. When plagued by intrusive thoughts relating to worst case scenarios in the near or distant future, use any combination of defusion strategies to label and dis-identify from them. For example, you might be ruminating over the thought that your boss will fire you because an email remained stuck in your Outbox after you left the office. You could choose to use defusion technique 1, which is labeling, and place the thought in the following template:

"I am noticing I am having the thought that my boss will fire me, and this is resulting in me feeling anxiety."

Again, this will not result in the anxiety vanishing, because that isn't the goal here – instead, you will not be worsening the situation by entering into the worry cycle or misconstruing the thought for reality. Next, you can think back to your values and engage in a corresponding values-based activity, or you can simply return your focus to the present moment, provided the activity in the present moment is not helping you to avoid the source of the anxiety. Doing this repeatedly will help facilitate change by communicating to your brain that these thoughts and concerns are illegitimate, thereby reducing the likelihood and intensity of their reemergence. More to the point, you will be learning to engage in your value-based life path regardless of the quality of your thoughts and emotions.

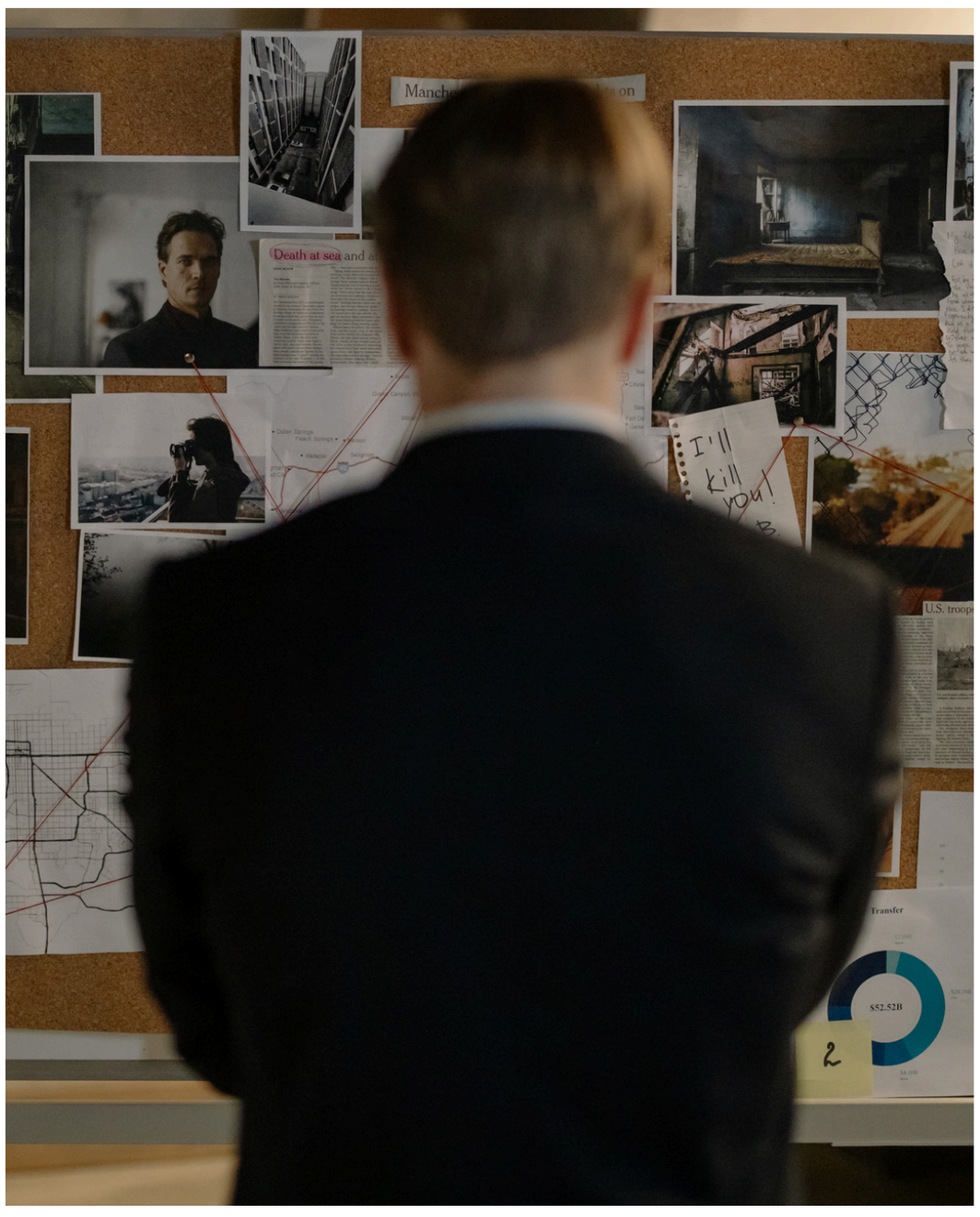
In summary, by accepting all fearful thoughts and choosing to step into uncertainty with values-based action, you will begin to develop a deep trust in your abilities by proving to yourself that you can, in fact, successfully navigate life's inherent unpredictability.

Positive beliefs about worry

Another major characteristic that maintains the worry cycle is a set of positive beliefs about the value and need for worry. Individuals who experience generalised anxiety tend to hold worrying in high regard, and these beliefs are not typically conscious until they are brought to the surface. Take a moment to consider your own beliefs about worry. What sort of value do you believe worrying has? Does it positively or negatively impact your life? And what would it mean if you were to stop worrying? What sort of messages did you receive about worrying throughout your early development? Note down your responses so you can consider them in greater detail. Typical beliefs held by those experiencing generalised anxiety might include:

- **"Worrying helps me to solve problems."**
- **"Worrying keeps me prepared."**
- **"Worrying shows I care."**

Let's take the first belief as an example - "Worrying helps me to solve problems." To evaluate the accuracy of this (or any other) belief, it is vital that we take on the role of a detective or curious scientist as we weigh up the evidence for and against our convictions. After we have collected the data, we can choose to continue to believe as we have done or change our beliefs in line with the new evidence. In the example template below, the evidence for and against the sample belief is provided. A blank template is available in the appendix for **Module 3**, which you can use to evaluate your own beliefs.



Belief: ***"Worrying helps me to solve problems."***

Evidence for	Evidence against
• I feel like I'm better prepared if I worry (but I am always prepared for things that never happen, and it takes a lot of energy) • Bad things rarely happen because I worry (what evidence is there that this is the case? Maybe bad things don't happen because they're so unlikely) • Worrying ensures I don't miss anything (but worrying means I miss out on a lot of things - I can't go to certain places)	• Worry makes me focused on the problem (I am so focused on what might happen that I forget to deal with what does actually happen) • Worry makes it almost impossible for me to take action (I am so afraid of what might happen that I can't take any steps forward) • There have been times where I did not worry, and nothing bad happened (there have been many times actually, and I'm always so surprised)

In the above example, it is clear that the evidence 'for' the belief does not hold up to further scrutiny. This is a great example of just how fickle some of our belief systems are, and that all that is required is a curious and courageous examination to determine how they stack up against the facts. The individual believing that worry held considerable value was able to see that their convictions were largely unfounded, leaving them the choice to either update their beliefs, or continue believing as they had.

Examining the evidence 'against' provides a number of interesting insights, namely that not worrying did ***not*** result in any feared catastrophe occurring. Furthermore, it was revealed that worrying actually inhibited the individual from taking positive action.

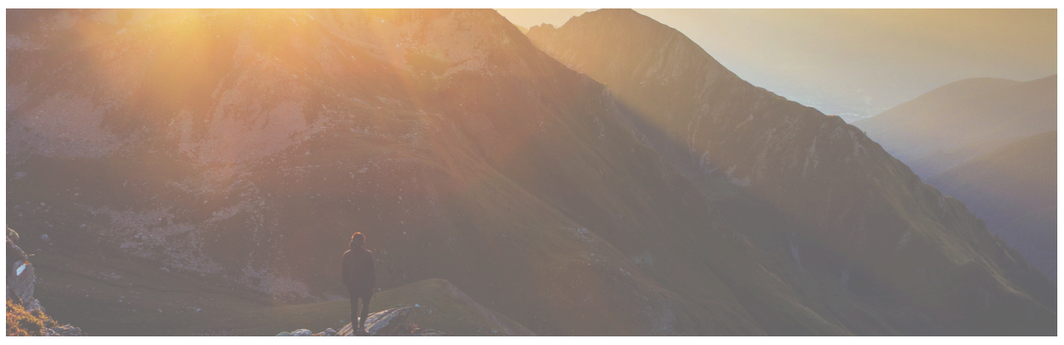
Once you have determined the evidence, and have critiqued it closely, formulate new beliefs that correspond more closely to the facts. For example, the belief ***"worrying helps me to solve problems"*** might be modified to ***"worrying isn't helpful and gets in the way of taking action."***

Each time you find yourself tempted to fall into the worry cycle, take a moments pause and think back to the lack of evidence regarding worry's effectiveness, and the newly formed belief(s) you developed as a result of this exercise. Because this is a process of habit change, it will require time to take effect.

Consistency is key!

Facts vs. Worry

Another exercise to assist with demonstrating the limited function of worry is the *Facts vs. Worry Exercise*, in which you analyse a feared situation that came to pass and evaluate whether your worry was justified or ultimately unnecessary given the outcomes. A blank exercise template will be available in the **Module 3** appendix.



Facts vs. Worry

What was/were the situations or circumstances causing you to worry?

I forgot to send an email to my boss as requested.

What did your worry believe would happen?

I was worried that I would be fired from my job. I couldn't sleep and kept thinking about it. I couldn't stop worrying and needed to constantly ask my partner whether they thought I would be fired or not.

What did happen? List the facts

I approached my boss the next day apologised for the delay in sending the email. He did not seem to mind and told me not to worry about it.

What does this comparison say about the need and value of worry?

That my worry was wrong, and that there was no need for it. My worry is not always telling the truth and it is often wrong.

Worry period

The last exercise included to address excessive worry is the designated ***worry period.*** This will be a period of time throughout your day that is given a strict start time, finish time, and duration, during which you will permit yourself as much worry and rumination as is required. As worries mount throughout your day, note them down and postpone them until the worry period, and use values-based activities to help redirect your focus away from the immediate impulse to worry further.

The worry period might be as follows:

- **6:00 - 6:20pm each night in the lounge room**

This means that ALL worry throughout your day must be postponed to this worry period.

This may seem strange, or even difficult initially, but the point of the exercise is to develop the skill and experience of not having to submit to the impulse to worry each time it arises. You will learn that you do have a greater degree of control over your worry than you may have previously thought.

When starting out, try and provide yourself with a worry period each day. As your tolerance and confidence increase, see if you can't designate a worry period once every 2 days. Increasing the time between worry periods will continue to develop your capacity to postpone worry further and further down the line, and you may find that you eventually have no need to return to your worry once the initial impulse passes!

MODULE 3 SUMMARY

In the final module of the guidebook we explored worry in great detail, including how it develops, what maintains the cycle and the many ways in which you can steadily relieve yourself from its control over your life.

A low tolerance for uncertainty was shown to maintain many of the core symptoms of general anxiety, and strategies for were provided for building a healthier relationship with the natural unpredictability of life.

You were encouraged to investigate your own beliefs about the worry, and the ways in which they might be contributing to symptoms. After engaging in the various belief modification tasks, you should now have a better understand of how and why your previous beliefs about worry were outdated and contrary to the available evidence. In most cases, excessive worry is simply at odds with reality, which is great news - life is not as scary or threatening as our brains would have us believe!

MODULE 3 APPENDIX

Evidence for and against table

Belief:

Evidence for	Evidence against

Facts vs. worry table

Facts vs. Worry
What was/were the situations or circumstances causing you to worry?
What did your worry believe would happen?
What did happen? List the facts
What does this comparison say about the need and value of worry?

CONGRATULATIONS!

You have made to the end of the Self-Therapy Guidebook for Generalised Anxiety! This is no small feat, as challenging our habits, some of which have been lifelong, is an arduous and exhausting process, but one that is ultimately rewarding and inspiring. While this guidebook was intended to be read from start to finish as a simulation of a course of therapy administered by a professional, it is highly recommended that you return to any section of the content as needed to maintain upkeep of your emerging recovery skillset.

As a final note, any process of personal change is hard, and requires reserves of willpower, disciplined action, and a clear goal or 'why' for undertaking it. Mental health, like anything else, is largely a composite of healthy micro-behaviours conspiring to improve your overall quality of life, so take it slow, and steadily build the behaviours necessary to facilitate this process.

Once again, congratulations for taking steps forward to improve your life, and we wish you the very best on the road to recovery from generalised anxiety.

The Greylight Psychology team